DRUGS · BLEEDING AND HAEMOSTASIS

Silver nitrate

Silver nitrate applied on a stick cauterises tissue chemically. It is the standard way to treat a visible bleeding point or prominent vessels on the anterior septum in epistaxis, and it shrinks granulation tissue in ear cavities, at tracheostomy stomas and in the nose. It needs a dry field to work, and cauterising both sides of the septum at the same level risks perforation.

1 CHAPTER ONE At a glance

The agent on one page.

1.1 Summary

SILVER NITRATE

	SUMMARY
Preparations	Applicator sticks with a tip of silver nitrate (commonly 75% with potassium nitrate)
How it works	Silver ions denature proteins and form a coagulum (eschar) on contact with moisture
ENT uses	Anterior epistaxis from a visible vessel; prominent vessels in recurrent childhood epistaxis; granulation tissue in mastoid cavities, ear canals, tracheostomy stomas and wounds; small bleeding points
Technique	Anaesthetise and decongest; dry the area; touch the stick to the vessel and roll it around the bleeding point, starting at the periphery and working inward; a few seconds of contact only
Avoid	Cauterising opposing areas of both sides of the septum at one sitting; contact with skin and eyes

2 CHAPTER TWO Using it in ENT

Practice.

2.1 Epistaxis

After topical co-phenylcaine or lidocaine with a decongestant, identify the vessel with a headlight or endoscope and suction. Cauterise the bleeding point; if both sides need treatment, wait 4 to 6 weeks between sides. Afterwards, an antiseptic cream (such as Naseptin, if no nut allergy) or petroleum jelly for 1 to 2 weeks helps the eschar heal.

2.2 Granulation tissue

Apply to granulations in a dry cavity or stoma, then treat the cause (infection, foreign body, a tube rubbing).

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

Pain and stinging (adequate anaesthesia); black staining of skin; chemical burns of the alar skin from run-off; septal perforation from bilateral or excessive cautery; failure if the field is wet.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Technique and why the field must be dry.
- Why not both sides of the septum at once.

4.2 Common pitfalls

COMMON PITFALLS

- Trying to cauterise an actively bleeding point without first controlling it with pressure and vasoconstrictor.

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